Name	: **Ms. Renuka Kelkar**		
PID No.	: MOL10046827	**Register On**	: 30/03/2024 10:50 AM
SID No.	: 924008456	**Collection On**	: 30/03/2024 11:08 AM
Age / Sex	: 37 Year(s) / Female	**Report On**	: 30/03/2024 5:00 PM
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Ref. Dr	: **DR. SATEESHCHANDRA M. DHADAKE**		

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HAEMATOLOGY

Complete Blood Count With - ESR

Haemoglobin (EDTA Blood/*Spectrophotometry)*	**11.4**	g/dL	12.5 - 16.0
Packed Cell Volume(PCV)/Haematocrit (EDTA Blood)	**35.0**	%	37 - 47
RBC Count (EDTA Blood)	4.64	mill/cu.mm	4.2 - 5.4
Mean Corpuscular Volume(MCV) (EDTA Blood)	**75.4**	fL	78 - 100
Mean Corpuscular Haemoglobin(MCH) (EDTA Blood)	**24.5**	pg	27 - 32
Mean Corpuscular Haemoglobin concentration(MCHC) (EDTA Blood)	32.5	g/dL	32 - 36
RDW-CV (EDTA Blood)	14.1	%	11.5 - 16.0
RDW-SD (EDTA Blood)	**38.1**	fL	39 - 46
Total Leukocyte Count (TC) (EDTA Blood)	6900	cells/cu.mm	4000 - 11000

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Basophils (EDTA Blood)	0.4	%	00 - 02

INTERPRETATION: Tests done on Automated Five Part cell counter. All abnormal results are reviewed and confirmed microscopically.

Absolute Neutrophil count (EDTA Blood)	4.7	10^3 / µl	1.5 - 6.6
Absolute Lymphocyte Count (EDTA Blood)	1.7	10^3 / µl	1.5 - 3.5
Absolute Eosinophil Count (AEC) (EDTA Blood)	0.1	10^3 / µl	0.04 - 0.44
Absolute Monocyte Count (EDTA Blood)	0.5	10^3 / µl	< 1.0
Absolute Basophil count (EDTA Blood)	0.0	10^3 / µl	< 0.2
Platelet Count (EDTA Blood)	368	10^3 / µl	150 - 450
MPV (EDTA Blood)	8.6	fL	8.0 - 13.3
PCT (EDTA Blood/*Automated Blood cell Counter*)	**0.315**	%	0.18 - 0.28
ESR (Erythrocyte Sedimentation Rate) (EDTA Blood)	**41**	mm/hr	< 20

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BIOCHEMISTRY

Glucose Fasting (FBS) (Plasma - F/*GOD-PAP)*	85.67	mg/dL	Normal: < 100 Pre Diabetic: 100 - 125 Diabetic: >= 126

INTERPRETATION: Factors such as type, quantity and time of food intake, Physical activity, Psychological stress, and drugs can influence blood glucose level.

Blood Urea Nitrogen (BUN) (Serum/*Urease UV / derived)*	**6.9**	mg/dL	7.0 - 21
Urea (Serum/*Urease/GLDH)*	**14.32**	mg/dL	15 - 45
Creatinine (Serum/*Modified Jaffe)*	**0.58**	mg/dL	0.6 - 1.1

INTERPRETATION: Elevated Creatinine values are encountered in increased muscle mass, severe dehydration,Pre-eclampsia, increased ingestion of cooked meat, consuming Protein/ Creatine supplements, Diabetic Ketoacidosis, prolonged fasting, renal dysfunction and drugs such as cefoxitin ,cefazolin, ACE inhibitors ,angiotensin II receptor antagonists,N-acetylcyteine, chemotherapeutic agent such as flucytosine etc.

Uric Acid (Serum/*Enzymatic)*	3.51	mg/dL	2.6 - 6.0
Calcium (Serum/*Arsenazo with ATCS)*	9.51	mg/dL	8.8 - 10.6

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BIOCHEMISTRY

Liver Function Test

Bilirubin(Total) (Serum/*DCA with ATCS)*	0.46	mg/dL	0.1 - 1.2
Bilirubin(Direct) (Serum/*Diazotized Sulfanilic Acid)*	0.19	mg/dL	0.0 - 0.3
Bilirubin(Indirect) (Serum/*Derived)*	0.27	mg/dL	0.1 - 1.0
SGOT/AST (Aspartate Aminotransferase) (Serum/*Modified IFCC)*	13.38	U/L	5 - 40
SGPT/ALT (Alanine Aminotransferase) (Serum/*Modified IFCC)*	7.89	U/L	5 - 41
GGT(Gamma Glutamyl Transpeptidase) (Serum/*IFCC / Kinetic)*	12.86	U/L	< 38
Alkaline Phosphatase (SAP) (Serum/*Modified IFCC)*	92.7	U/L	42 - 98
Total Protein (Serum/*Biuret)*	7.19	gm/dl	6.0 - 8.0
Albumin (Serum/*Bromocresol green)*	4.60	gm/dl	3.5 - 5.2
Globulin	2.59	gm/dL	2.3 - 3.6

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Lipid Profile

Cholesterol Total (Serum/*CHOD-PAP with ATCS)*	197.07	mg/dL	Optimal: < 200 Borderline: 200 - 239 High Risk: >= 240
Triglycerides (Serum/*GPO-PAP with ATCS)*	76.70	mg/dL	Optimal: < 150 Borderline: 150 - 199 High: 200 - 499 Very High: >= 500

INTERPRETATION: The reference ranges are based on fasting condition. Triglyceride levels change drastically in response to food, increasing as much as 5 to 10 times the fasting levels, just a few hours after eating. Fasting triglyceride levels show considerable diurnal variation too. There is evidence recommending triglycerides estimation in non-fasting condition for evaluating the risk of heart disease and screening for metabolic syndrome, as non-fasting sample is more representative of the "usual" circulating level of triglycerides during most part of the day.

HDL Cholesterol (Serum/*Immunoinhibition)*	**47.99**	mg/dL	Optimal(Negative Risk Factor): >= 60 Borderline: 50 - 59 High Risk: < 50
LDL Cholesterol (Serum/*Calculated)*	133.8	mg/dL	Optimal: < 100 Above Optimal: 100 - 129 Borderline: 130 - 159 High: 160 - 189 Very High: >= 190
VLDL Cholesterol	15.3	mg/dL	< 30

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INTERPRETATION: 1.Non-HDL Cholesterol is now proven to be a better cardiovascular risk marker than LDL Cholesterol.
2.It is the sum of all potentially atherogenic proteins including LDL, IDL, VLDL and chylomicrons and it is the "new bad cholesterol" and is a co-primary target for cholesterol lowering therapy.

Total Cholesterol/HDL Cholesterol Ratio (Serum/*Calculated)*	4.1	Optimal: < 3.3 Low Risk: 3.4 - 4.4 Average Risk: 4.5 - 7.1 Moderate Risk: 7.2 - 11.0 High Risk: > 11.0
Triglyceride/HDL Cholesterol Ratio (TG/HDL) (Serum/*Calculated)*	1.6	Optimal: < 2.5 Mild to moderate risk: 2.5 - 5.0 High Risk: > 5.0
LDL/HDL Cholesterol Ratio (Serum/*Calculated)*	2.8	Optimal: 0.5 - 3.0 Borderline: 3.1 - 6.0 High Risk: > 6.0

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Glycosylated Haemoglobin (HbA1c)

HbA1C (Whole Blood/*HPLC)*	5.1	%	Normal: 4.5 - 5.6 Prediabetes: 5.7 - 6.4 Diabetic: >= 6.5

INTERPRETATION: If Diabetes - Good control : 6.1 - 7.0 % , Fair control : 7.1 - 8.0 % , Poor control >= 8.1 %

Estimated Average Glucose (Whole Blood*)*	99.67	mg/dL

INTERPRETATION: Comments
HbA1c provides an index of Average Blood Glucose levels over the past 8 - 12 weeks and is a much better indicator of long term glycemic control as compared to blood and urinary glucose determinations.
Conditions that prolong RBC life span like Iron deficiency anemia, Vitamin B12 & Folate deficiency, hypertriglyceridemia,hyperbilirubinemia,Drugs, Alcohol, Lead Poisoning, Asplenia can give falsely elevated HbA1C values.
Conditions that shorten RBC survival like acute or chronic blood loss, hemolytic anemia, Hemoglobinopathies, Splenomegaly,Vitamin E ingestion, Pregnancy, End stage Renal disease can cause falsely low HbA1c.

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IMMUNOASSAY

THYROID PROFILE / TFT

T3 (Triiodothyronine) - Total (Serum/*ECLIA)*	1.33	ng/ml	0.7 - 2.04

INTERPRETATION:
Comment :
Total T3 variation can be seen in other condition like pregnancy, drugs, nephrosis etc. In such cases, Free T3 is recommended as it is Metabolically active.

T4 (Tyroxine) - Total (Serum/*ECLIA)*	10.17	µg/dl	4.2 - 12.0

INTERPRETATION:
Comment :
Total T4 variation can be seen in other condition like pregnancy, drugs, nephrosis etc. In such cases, Free T4 is recommended as it is Metabolically active.

TSH (Thyroid Stimulating Hormone) (Serum/*ECLIA)*	1.96	µIU/mL	0.35 - 5.50

INTERPRETATION:
Reference range for cord blood - upto 20
1 st trimester: 0.1-2.5
2 nd trimester 0.2-3.0
3 rd trimester : 0.3-3.0
(Indian Thyroid Society Guidelines)
Comment :
1.TSH reference range during pregnancy depends on Iodine intake, TPO status, Serum HCG concentration, race, Ethnicity and BMI.

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SPECIAL CHEMISTRY

Vitamin D (25-Hydroxy Vit D) (Serum/*ECLIA)*	39.52	ng/ml	Sufficiency: 31.0 - 100.0 Insufficiency: 21.0 - 30.0 Deficiency: < 20.0 Toxicity: > 100.0

INTERPRETATION:
Comments:
Vitamin D refers to a group of fat-soluble secosteroids responsible for enhancing intestinal absorption of calcium, iron, magnesium,phosphate and zinc. Vitamin D includes D3 (Cholecalciferol) and D2 (Ergocalciferol). Vitamin D3 is formed in the skin in response to exposure to UVB from natural sunlight, or is ingested. Vitamin D2 mainly comes from plant sources. Vitamin D3 and D2 are hydroxylated in the liver to 25-hydroxyvitamin D (25-OHD)/Calcidiol.
This is the major circulating form of vitamin D and is the target for assays measuring vitamin D status.

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